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Name: بدور ابراهيم حسن ابراهيم National ID: 26104290101369	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms

هزمتنا فيمارا





Patient Name: Budor Ibrahim Hassan Ibrahim	Date: 31/05/2023
Exam: ultrasound BREAST (BOTH SIDE)	Age: 52 Year(s)
Medical ID: 854/2023	Pat. ID: 30027

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with operative bed seroma measures 2.4 x 7 cm, at 3 o'clock showing internal septations. Other smaller collection is also noted at axillary operative bed measures 0.8 x 2.4 cm. no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with operative bed seroma. No evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Aya Deabes

31/05/2023 4:20 PM

Name: بدور ابراهيم حسن		
MR. : 4231		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 64-year-old female with a history of left breast invasive ductal carcinoma, grade II, ER 8/8, PR 8/8, HER2-negative, staged T2N1M0. She underwent left wide local excision with left axillary lymph node dissection in April 2023, with pathology showing 1/23 positive nodes. Adjuvant chemotherapy consisted of AC (doxorubicin and cyclophosphamide) for 4 cycles followed by 12 cycles of paclitaxel. She has been receiving adjuvant aromatase inhibitor therapy since February 2024 for secondary prevention. Current imaging includes a chest X-ray, and laboratory workup is complete with normal calcium levels. The patient has completed 4 cycles of AC and 12 cycles of Taxol.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
	أيه مدحت بدوي محمد	الطبيب المعالج :
يعتمد ,		



Patient Name: Budor Ibrahim Hassan Ibrahim	Date : 26/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 63 Year(s)
Medical ID: 854/2023	Pat. ID: 30027

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

26/12/2024 2:28 PM



Patient Name : الأستاذة / بدور ابراهيم حسن ابراهيم	Request No. : 13007332
Patient No. : 26104290101369	Request Date : 13/04/2023
Path No. : 1160/2023	Verification Date : 30/04/2023
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed you

Pathology Report

Clinical Data:

Left breast mass , Upfront surgery

Nature of the specimen:

Left WLE & Left ALND

Gross Examination:

- Procedure: Left WLE specimen measured 11x8x3 cm , Covering skin ellipse measured 6x2.8 cm .
- Tumor extent : An ill defined greyish white firm mass measured 2.5x2.5x2 cm .
- Distance from margins: Away from lateral margin (Least margin) 0.3 cm .
- Lymph nodes number and size: A fibrofatty tissue piece measured 7x5.5x3.5 cm enclosing 23 LNs up to 2x1.5 cm
- Frozen section: Free margins (microcalcification in inferior margin)

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Lymph node involvement: Positive metastatic deposit in one LN (1/23) , with capsular invasion
- Resection Margins: Free margins

Diagnosis:

Left breast mass , Left WLE & Left ALND :

- Invasive duct carcinoma grade II
- Free margins
- Positive metastatic deposit in one LN (1/23) , with capsular invasion
- Stage : pT2 , pN1

Prof.Dr.Gamal Abdelatty

Verified by :

dr.esraa magdy

Printing Date : 01/05/2023 10:11:31

Page 1 of 2

Printing By : marwa mohamed youss



Patient Name: Budor Ibrahim Hassan Ibrahim	Date: 14/12/2023
Exam: C.T ABDOMEN & PELVIS	Age: 62 Year(s)
Medical ID: 854/2023	Pat ID: 30027

NON CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Reported case left breast carcinoma underwent left BCS, compared to last study dated 12/03/2023 the recent study shows

1. Chest:

- Both breasts are partially out of field
- left breast skin thickening likely post operative sequel
- no pathologically enlarged axillary lymphadenopathy
- still noted lingual atelectatic band.
- No sizable pulmonary nodules could be seen.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Enlarged thyroid lobe with multiple hypoechoic nodules and retro sternal extension more evident on left lobe... for US correlation.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with calcific foci ... likely old granulomatous infection
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- Rather stationary course of Anterior abdominal wall soft tissue density
- No abdominal or pelvic masses or collections.